National Cancer Control Programme (NCCP) 00350 · clinician draft

Pertuzumab + trastuzumab · with selected chemotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Combination visit · select the chemotherapy

Active medicines: Pertuzumab + Trastuzumab + Separately selected chemotherapy

Emetogenic risk: Determined by the selected chemotherapy

Chemotherapy has not been specified here. A complete supportive prescription cannot be generated from this antibody framework alone.

Additional phase-specific support

- Open the exact companion National Cancer Control Programme (NCCP) regimen before prescribing. Its antiemetics, premedication, granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), blood thresholds and phase schedule apply.
- Antibody-specific premedication is not usually needed; consider paracetamol/antihistamine for the reaction history.
- If an anthracycline phase is selected, finish it before starting these antibodies with the taxane.
- Confirm indication and current access: NCCP00350b marks adjuvant pertuzumab reimbursement as not approved.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00350 v3 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.16 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Antibodies only · if separately prescribed

Active medicines: Pertuzumab + Trastuzumab

Emetogenic risk: Minimal for antibodies only

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Confirm that chemotherapy has actually ended or is separately held; do not apply this minimal-risk script to a combination visit.
- No routine cytotoxic antiemetic, taxane premedication or chemotherapy granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is carried forward.
- Continue cardiac checks every 12 weeks and at completion; a phase change does not reset the clock.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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